Find Out Who Will Really Be There Before Your Husband's Care Changes

Four prompts to lay out when help is needed hour by hour, check who is truly available, find the overnight and backup gaps, and write down which worries belong to a professional.

Where you are

Your husband is being discharged, or moving to a new arrangement, within days. Everyone says they'll pitch in. You have a feeling the plan rests on you being awake at 3 a.m. and on a daughter who lives two hours away.

This guide gives you four ChatGPT prompts to organize what you already know and to see what you don't know yet. It is preparation, not an assessment. Use only information you have permission to share, with names removed. The AI organizes and drafts; it does not diagnose, assess safety, or decide. Bring clinical questions to a clinician, and call emergency services for an emergency.

What you'll have at the end

You can show, on one page, when he will need someone, who has actually said yes, where the gaps are, and which concerns you are taking to the care team rather than settling yourself.

A one-page coverage review: When help is needed, Who has committed, Gaps and backup, Concerns for the care team, and Who follows up on each.

Before you start

- Whatever the hospital or current team has told you about the discharge or move, even if it's only a date.
- The names of everyone who has offered to help.
- Your own honest sense of what you can do and for how long. Rough answers are fine; you don't need a schedule yet.

PROMPT 1

Lay out when help will be needed

WHY THIS MATTERS

A list of what he needs isn't the same as a clock. Getting up at night, the first hour of the morning, and meals are when help is needed most, and they are the hardest hours to fill.

WHAT YOU'LL NEED

What you know about what he will need help with, from the discharge team or from watching him.

COPY THIS PROMPT

> My husband is moving to a new care arrangement soon. Here is what I know about what he needs help with, and what the hospital or care team has told me:

> [PASTE WHAT YOU KNOW]
>
> Build me a 24-hour timeline for an ordinary day in blocks: EARLY MORNING, MORNING, MIDDAY, AFTERNOON, EVENING, OVERNIGHT. In each block, list the help he will likely need based only on what I wrote, and mark whether someone needs to be present, nearby, or just reachable. Where I haven't given you enough to say, write “unknown” rather than guessing. Do not add care tasks no professional has mentioned, and do not change or interpret any instructions about medication.

WHAT TO LOOK FOR

The overnight block and the “unknown” entries.

WHAT TO DO NEXT

Take the unknowns to the discharge planner or nurse before the move date.

PROMPT 2

Check who is really available

WHY THIS MATTERS

“I'll help” and “I can be there Tuesdays and Thursdays from 9 to 2” are different things. The first is kindness. The second is coverage.

WHAT YOU'LL NEED

Everyone who has offered, and the specifics they have actually given you, if any.

COPY THIS PROMPT

> Here are the people who have said they'll help with my husband's care, with whatever details they've given me:
>
> [PASTE YOUR LIST]
>
> Make a table with columns: WHO, WHAT THEY'VE OFFERED, DAYS AND TIMES THEY'VE CONFIRMED, HOW LONG THEY CAN KEEP THIS UP, and WHAT I STILL NEED TO ASK THEM. Treat a general offer as unconfirmed. Then place only the confirmed help onto this 24-hour timeline and show me every block that has nobody in it:
>
> [PASTE THE TIMELINE FROM PROMPT 1]

WHAT TO LOOK FOR

How much of the plan is still a general offer.

WHAT TO DO NEXT

Call each person and turn their offer into days and times, or into an honest no.

PROMPT 3

Be honest about what you can carry

WHY THIS MATTERS

The plan usually assumes the main caregiver can do whatever is left over. If that's you, your limits are part of the safety picture, not a weakness in it.

WHAT YOU'LL NEED

A frank note about your own health, sleep, work, and how long you could keep up the current pace.

COPY THIS PROMPT

> I'm the main caregiver for my husband. Here is an honest description of my own situation: my health, my sleep, my work and other responsibilities, and what I've managed so far:
>
> [PASTE YOUR NOTES]
>
> And here is the coverage table with the empty blocks:
>
> [PASTE THE TABLE FROM PROMPT 2]
>
> Tell me plainly which empty blocks the plan is silently expecting me to fill, and add them up in hours per week. Then list the signs, from what I've written, that this would be hard to sustain for more than a few weeks. Don't reassure me and don't tell me what I should do. Just show me the load.

WHAT TO LOOK FOR

The number of hours. Say it out loud to someone.

WHAT TO DO NEXT

Decide which of those hours you are genuinely willing to take, and which need another answer.

PROMPT 4

Sort the gaps you can fill from the ones a professional must see

WHY THIS MATTERS

Some gaps are solved by a phone call to a cousin. Others, like a fall risk or a medication he can't manage alone, are not yours to settle. The care team needs to hear those before the move, not after.

WHAT YOU'LL NEED

Everything from the first three prompts, plus any safety worries you have noticed or been told about.

COPY THIS PROMPT

> Here is what I've worked out about support for my husband's next care arrangement, plus the safety worries I have or have been told about:
>
> [PASTE EVERYTHING FROM PROMPTS 1 TO 3 AND YOUR WORRIES]
>
> Put it all on one page under five headings: WHEN HELP IS NEEDED, WHO HAS COMMITTED, GAPS AND BACKUP, CONCERNS FOR THE CARE TEAM, and WHO FOLLOWS UP ON EACH. Under CONCERNS FOR THE CARE TEAM, place anything about falls, medication, supervision, or the home that needs a professional's judgment, written as a question for them. Do not answer those questions, do not say whether the arrangement is safe, and do not suggest equipment or changes to his care.

WHAT TO LOOK FOR

A page where every gap has a name beside it, and the clinical items are questions, not conclusions.

WHAT TO DO NEXT

Send the CONCERNS section to the discharge planner or care coordinator before the move. Keep the whole page for the next step.

Get the whole picture before the move

Your coverage map shows the hours. The care interview adds what he can and can't do during them, who is caring for him, and what the care team has already said, so your care consultant is looking at a full Family Care Profile when she reviews it.

Create Your Free Family Care Profile ›